BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

Check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver	Baseline bloods; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Blood pressure / urine / wounds	blood pressure (BP), urine protein and wound/procedure review	Blood pressure before each cycle AND after treatment; urine protein each cycle	Report bleeding, severe headache or abdominal pain urgently	Additional urine tests and specialist review if abnormal	Confirm wound-healing and procedure plan
Neuropathy / cold sensitivity	Record existing nerve symptoms	Formal neuropathy evaluation every two cycles; review symptoms sooner if they occur	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review
Heart checks	Review cardiac history; electrocardiogram (ECG) when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment
international normalised ratio (blood-clotting test) (INR) only if taking warfarin	Tell the team about warfarin and arrange a baseline INR	INR weekly until the warfarin dose is stable, then before each cycle	Report bleeding or unusual bruising	Only change anticoagulants as instructed	Continue the individually agreed anticoagulation follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00446 v7b · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Modified FOLFOX-6 + bevacizumab: treatment and checks

Metastatic colorectal cancer

Day 1: bevacizumab, oxaliplatin, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00446	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00446	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Blood pressure / urine / wounds NCCP 00446	BP, urine protein and wound/ procedure review	Blood pressure before each cycle AND after treatment; urine protein each cycle	Report bleeding, severe headache or abdominal pain urgently	Additional urine tests and specialist review if abnormal	Confirm wound-healing and procedure plan

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Neuropathy / cold sensitivity NCCP 00446	Record existing nerve symptoms	Formal neuropathy evaluation every two cycles; review symptoms sooner if they occur	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review
Heart checks NCCP 00446	Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans NCCP 00446	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment
INR only if taking warfarin NCCP 00446	Tell the team about warfarin and arrange a baseline INR	INR weekly until the warfarin dose is stable, then before each cycle	Report bleeding or unusual bruising	Only change anticoagulants as instructed	Continue the individually agreed anticoagulation follow-up

What each check is for

1

14-day treatment cycle

Day ward → home pump → review

Modified FOLFOX-6 + bevacizumab is used for cancer of the colon or rectum that has spread. Treatment aims to control the cancer; the likely benefit and review plan depend on your individual situation. Surgery may be reconsidered in selected cases after multidisciplinary review.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

Sources: NCCP 00446

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.33